

PO No :PO10000933382-172



Customer Name	: Mr.PRIYANK GUPTA	Collected Via	: TATA 1MG OKHLA
Age/Gender	: 51/Male	Referred By	: Dr.
Lab Visit ID	: OKH2687924	Collection Date	: 05/Jan/2026 08:45AM
Barcode ID/Order ID	: D26242225 / 15377109	Report Date	: 05/Jan/2026 02:36PM
Sample Type	: Serum	Report Status	: Final Report

BIOCHEMISTRY

Test Name	Result	Unit	Bio. Ref. Interval	Method
Lipid Profile				
Cholesterol - Total	152	mg/dL	Low (desirable): < 200 Moderate (borderline) 200-239 High: >= 240	Enzymatic
Triglycerides	91	mg/dL	Normal: <150, Borderline: 150 - 199, High:200-499, Very High>=500	GPO
Cholesterol - HDL	44	mg/dL	High risk <40mg/dL Low risk >=40mg/dL	Cholesterol Esterase
Cholesterol - LDL	90	mg/dl	Desirable: <100 Above desirable: 100 - 129 Borderline high : 130 - 159 High : 160 - 189 Very high : >=190	Calculated
Cholesterol- VLDL	18	mg/dl	<30	Calculated
Cholesterol : HDL Cholesterol	3.4	Ratio	Desirable : 3.5-4.5 High Risk : >5	Calculated
LDL : HDL Cholesterol	2.03	Ratio	Desirable : 2.5-3.0 High risk : >3.5	Calculated
Non HDL Cholesterol	108	mg/dL	Desirable:< 130, Above Desirable:130 - 159, Borderline High:160 - 189, High:190 - 219, Very High: >= 220	Calculated

Comment:

- Lipid results show analytical and biological variation; repeat testing may be recommended before diagnosis or treatment

NABL certificate
and scope

This test has been performed at

TATA 1MG OKHLAAddress: 2nd Floor, B-225, Okhla Phase I,
Okhla Industrial Estate, New Delhi, Delhi
110020
Dr. Dhananjay Singh
MBBS, MD(Pathology)
Consultant Pathologist
Reg No: 63325Scan for
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decisions.

- Indians lie at high risk of developing early (a decade earlier than western populations) and more severe cardiovascular disease (ASCVD); higher mortality. Dyslipidemia (abnormal lipid profile) affects nearly 80% of population.
- **Total cholesterol** is the sum of all cholesterol in the blood, including HDL, LDL, VLDL, and remnants.
- **LDL Cholesterol (LDL-C)**, is the main "bad" cholesterol that contributes to plaque buildup, increasing the risk of heart disease and stroke, typically calculated by the Friedewald formula. Direct LDL-C measurement by homogeneous enzymatic assays carried out when triglycerides >400 mg/dL or dysbetalipoproteinemia.
- **High-density lipoprotein (HDL)** or "good" cholesterol is anti-atherogenic (protective). Low HDL-C is a cardiovascular risk factor; seen in almost two-third of Indians. Values above 60 mg/dl are considered protective.
- **Triglyceride (TG)** are a key driver of CVD. Indians are especially prone to atherogenic dyslipidemia—high TG, low HDL-C, and high LDL-C—closely linked to diabetes, metabolic syndrome, and insulin resistance; making TG management crucial.
- **Non-HDL-Cholesterol (non-HDL-C)** Non-HDL-C measures all plaque-forming lipoproteins and is vital to monitor in high-TG patients (e.g., diabetics, obese) and those on statin therapy.
- **Lipid Association of India (LAI-2020) recommends:-**

- Screening of all Indians above the age of 20 years for CVD risk factors, esp. lipid profile.
- Identification of Major Risk factors: Age (male ≥45 years, female ≥55 years); Family h/o heart disease at younger age (<55 yrs in males, <65 yrs in female or before menopause), current smoking/tobacco use, High blood pressure, Low HDL (males <40 mg/dl and females <50mg/dl).
- Fasting not mandatory; both fasting and non-fasting lipid profiles are useful for screening in Indian patients.
- LAI identifies both LDL-C and non-HDL-C as risk factors and recommends LDL-C, non-HDL-C and Apo-B as targets of lipid-lowering therapy.
- Lifestyle changes are the first-line approach for managing and preventing dyslipidemia. Treatment in low-risk individuals is initiated only after 3 months of unsuccessful lifestyle modification.
- Testing for Apolipoprotein B(Apo-B), hsCRP, Lp(a) should be considered for patients in moderate risk group.

Treatment targets for lipid-lowering therapy for various ASCVD risk groups

Risk Group	Treatment targets		
	LDL-C, mg /dL (primary target)	Non-HDL-C, mg /dL (co-primary target)	Apo-B, mg /dL (secondary target)
Low-risk group	<100	<130	<90
Moderate-risk group	<100 (optional <70)	<130 (optional <100)	<90
High-risk group	<70	<100	<80
Very high-risk group	<50	<80	<65
Extreme-risk group- category A	<50 (optional ≤30)	<80 (optional ≤60)	<65
Extreme-risk group- category B	≤30	≤60	<50
Extreme-risk group- category C	10-15	40-45	-

Source: LAI (2024) Consensus Statement IV

- Per **NCEP Expert Panel (2011) guidelines**, universal dyslipidemia screening is advised at 9–11 years and repeated at 17–21 years. Screening before age 2 yrs is not recommended; from age 2 onward, selective screening is done for children with a family history of premature CVD or risk factors such as obesity, diabetes, or hypertension.

Note: Biological Reference Interval as per National Cholesterol Education Program (NCEP) ATP III and LAI guidelinesNABL certificate
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*** End Of Report ***

Disclaimer:

1. The reported results based on laboratory investigation, are only for the purposes of diagnosis and should be clinically correlated and interpreted by the referring physician/ medical practitioner. For any queries relating to the reported results, you may write to our customer support team on care@1mg.com
2. It is presumed that the tests performed are, on the specimen / sample being to the patient named or identified and the verifications of particulars have been confirmed by the patient or his / her representative at the point of generation of said specimen.
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4. The patient's details along with their results in certain cases like notifiable diseases and as per local regulatory requirements will be communicated to the assigned regulatory bodies.
5. The patient samples can be used as part of internal quality control, test verification, data analysis purposes within the testing scope of the laboratory.
6. This report is not valid for medico legal purposes. It is performed to facilitate medical diagnosis only.
7. Pregnant women should seek guidance from a qualified obstetrician as test parameters may vary during pregnancy

TATA 1mg Labs

NABL certificate
and scope

MC - 5424

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